

CI Institute of Nursing

CNA / CDPH Online CE Compiled Course Materials

CNA 24-hour online CE course using CCCCCO/NATP Modules 10-17 as the source backbone, delivered as 12 Moodle units.

LMS development may not start until Vanessa Valerio, RN or another approved SME has reviewed and approved the course materials. Internal SME approval is required before Aldrin/Dagny begin Moodle course-shell build, content import, quiz import, learner-access configuration, certificate setup, or production implementation. SME approval does not replace agency approval.

Course Materials Scope

CNA 24-hour online CE course using CCCCO/NATP Modules 10-17 as the source backbone, delivered as 12 Moodle units.

This compiled PDF is the lane-specific course-materials packet for end-user review. It is not an official agency approval claim and must remain Draft / Pending Approval until agency metadata is issued.

Source Content: 00_CNA_COURSE_CONTENT_INDEX.md

CNA Course Content Index

Status: Draft / Pending CDPH Approval **Source backbone:** CCCC/NATP Nurse Assistant Model Curriculum, Revised December 2018, Modules 10-17 **Output purpose:** Actual Moodle-ready learner course content for the CNA/CDPH CE lane.

Required Structure Validation

- ? CNA Moodle content created: yes
- ? Unit structure: 12 application-facing units x 2 online CE hours
- ? Total target time: 24 online CE hours
- ? Backbone used: CCCC/NATP Modules 10-17
- ? Old ContentV2 M00-M07 used as current: no
- ? Old 8 x 3 hour structure used: no
- ? Question bank count: 120 questions
- ? Government forms copied: no
- ? Signer binders copied: no
- ? Certificate issuance while approval pending: blocked

Files

File	Purpose
00_CNA_COURSE_CONTENT_INDEX.md	Folder index, validation summary, and build order.
01_CNA_24HR_12_UNIT_MOODLE_COURSE_CONTENT.md	Learner-facing Moodle-ready unit content and activity blocks.
02_CNA_24HR_12_UNIT_MOODLE_BUILD_WORKBOOK.xlsx	Moodle build workbook with course map, activities, question bank, completion gates, and validation.
03_CNA_QUESTION_BANK_MOODLE_IMPORT_READY.csv	Moodle import-ready question bank CSV with 120 questions.
04_CNA_COMPLETION_AND_CERTIFICATE_RULES.md	Completion, final assessment, attestation, and certificate gate rules.
05_CNA_ACTIVITY_AND_INTERACTION_MAP.md	Per-unit activity map and active learner interaction plan.
06_CNA_INSTRUCTOR_REVIEW_NOTES.md	SME/instructor/admin review notes and launch checks.

Moodle Build Order

1. Build Section 0 orientation/identity/no-PHI attestation.
2. Build U01-U12 exactly as 12 units with 2 online CE hours each.
3. Import the CSV question bank and create each unit knowledge check.
4. Create the final assessment using the course-level quiz bank subset.
5. Configure completion and restrict-access rules from 04_CNA_COMPLETION_AND_CERTIFICATE_RULES.md.
6. Keep certificate issuance disabled until approval status allows it.

Source Content:

01_CNA_24HR_12_UNIT_MOODLE_COURSE_CONTENT.md

CNA 24-Hour / 12-Unit Moodle Course Content

Status: Draft / Pending CDPH Approval **Source backbone:** CCCC/NATP Nurse Assistant Model Curriculum, Revised December 2018, Modules 10-17 **Course structure:** 12 application-facing units x 2 online CE hours each = 24 online CE hours total.

This file is written for a Moodle builder. Each unit contains the required Moodle activity blocks: Page: Unit Overview; Page: Lesson Content; H5P or Lesson Activity: Scenario/Application; Quiz: Knowledge Check; Assignment or Feedback: Reflection/Attestation; Completion Gate: Required activity completion.

Compliance posture: Do not advertise, issue certificates, or represent this course as approved until CDPH approval status, provider/course identifiers, and certificate wording are confirmed. Online CE only. No clinical hours, no simulation hours, no hands-on competency validation, no real resident information, no copied government forms, and no copied signer binders.

Course Map

Unit	Title	Hours	Source Backbone
U01	Vital Signs I: Measurement Foundations	2	Module 10 - Vital Signs
U02	Vital Signs II: Reporting and Abnormal Findings	2	Module 10 - Vital Signs
U03	Nutrition and Hydration	2	Module 11 - Nutrition
U04	Emergency Procedures	2	Module 12 - Emergency Procedures
U05	Long Term Care Patient/Resident I: Resident-Centered Care	2	Module 13 - Long Term Care Patient/Resident
U06	Long Term Care Patient/Resident II: Chronic Conditions and Daily Support	2	Module 13 - Long Term Care Patient/Resident
U07	Rehabilitative Nursing	2	Module 14 - Rehabilitative Nursing
U08	Observation and Charting I: Objective Observation	2	Module 15 - Observation and Charting
U09	Observation and Charting II: Documentation and Reporting	2	Module 15 - Observation and Charting
U10	Death and Dying	2	Module 16 - Death and Dying
U11	Patient/Resident Abuse I: Recognition and Prevention	2	Module 17 - Patient/Resident Abuse
U12	Patient/Resident Abuse II: Reporting and Response	2	Module 17 - Patient/Resident Abuse
Total	12 units	24	Modules 10-17

Start Here / Section 0

Moodle activities before U01: identity confirmation if required by the final approved packet, academic-integrity attestation, no-PHI agreement, technology orientation, and statement that this course is Draft / Pending CDPH Approval. Learners must attest that they will not enter real resident, patient, facility, coworker, or family information into any activity.

Final Assessment / Course-Level Quiz Bank Subset

Create a final Moodle quiz after U12 using a random subset from 03_CNA_QUESTION_BANK_MOODLE_IMPORT_READY.csv. Recommended settings: 50 questions selected across U01-U12, at least 4 questions from each unit when using a fixed build, 80 percent passing score, limited attempts per local policy, shuffled questions/answers, deferred feedback, and review options that do not expose a reusable answer key. Certificate access remains restricted until all unit gates, active participation, final assessment pass, identity/attestation, and approval-status gates are complete.

U01 - Vital Signs I: Measurement Foundations

Status: Draft / Pending CDPH Approval **Source backbone mapping:** Module 10 - Vital Signs (Objectives 1-4 and 15-16: terminology, purpose of vital signs, temperature, comfort measures within scope, pain observation, recording, and reporting.) **Seat-time target:** exactly 2 online CE hours (120 minutes). Suggested allocation: overview 10, lesson content 45, scenario/application 25, active interaction 15, knowledge check 15, reflection/attestation 10.

Moodle Activity Block - Page: Unit Overview

Purpose: This unit converts Module 10 - Vital Signs into learner-facing online CE content for CNA renewal support. It is online continuing education only and does not provide clinical hours, skills validation, or approval claims.

Learning objectives:

- ? Explain why temperature, pulse, respirations, blood pressure, and pain observations are collected as vital signs.
- ? Use vital-sign terminology accurately when reporting to licensed staff.
- ? Differentiate routine, abnormal, and reportable temperature and pain observations at the CNA level.
- ? Record vital-sign observations in factual, timely, scope-safe language.

Compliance reminder: Use only fictional information in activities. Do not enter real resident, patient, facility, coworker, or family information. The course remains Draft / Pending CDPH Approval.

Moodle Activity Block - Page: Lesson Content

Vital signs as early warning information

Vital signs help the care team see whether a resident is stable or changing. CNAs often notice the first clues because they are present during meals, hygiene, transfers, and routine care. This online CE teaches the knowledge and judgment around measurement foundations; it does not validate hands-on competency or award clinical hours.

Temperature foundations

Temperature reflects heat balance in the body. Terms such as afebrile, febrile, pyrexia, axilla, tympanic, Celsius, and Fahrenheit help staff communicate clearly. A CNA reports the reading, route, time, and related observations such as chills, sweating, flushed skin, confusion, or the resident saying they feel hot or cold.

Pain as a vital observation

Pain can affect pulse, respirations, blood pressure, sleep, mobility, appetite, and mood. The resident's report matters. CNAs use the assigned pain scale, observe nonverbal signs such as guarding or grimacing, and report without diagnosing or deciding medication needs.

Documentation and escalation

Accurate vital-sign reporting includes the number, method or site, time, resident position when relevant, and who was notified. If a finding is unusual, the CNA follows facility policy to recheck if directed and report promptly. Routine charting never replaces urgent reporting.

Builder note: Place this lesson content in a Moodle Page or Book page with completion tracking enabled for view plus minimum expected engagement time. Keep source-module names in the builder notes and do not brand or reproduce original CCCCO/NATP source pages as learner pages.

Moodle Activity Block - H5P or Lesson Activity: Scenario/Application

Scenario: Resident A is usually talkative at breakfast. Today Resident A is quiet, wrapped in two blankets, says 'I feel cold,' has an oral temperature of 100.8 F, and reports right knee pain as 6 out of 10.

Required learner task: H5P drag-and-drop: sort each observation into measure/record, report now, or do not chart as a diagnosis.

Feedback design: Each incorrect path should explain the CNA scope issue, reporting delay, privacy risk, or documentation problem. Correct paths should reinforce observe, assist, report to the licensed nurse, document within scope, and protect dignity.

Moodle Activity Block - Quiz: Knowledge Check

Settings: 10 auto-graded multiple-choice items, shuffled answers, unlimited formative attempts or local policy limit, passing score 80 percent, feedback shown after attempt. Use 03_CNA_QUESTION_BANK_MOODLE_IMPORT_READY.csv for import.

Internal answer/rationale preview:

Question	Correct	Rationale
U01_Q01_source	A	U01 is mapped to Module 10 - Vital Signs within the 12 unit x 2 hour CNA course.
U01_Q02_priority	B	The CNA observes, reports, documents, and follows policy within scope.
U01_Q03_scope	C	The listed unsafe action exceeds CNA role or violates course guardrails.
U01_Q04_charting	D	Objective documentation records facts, statements, observations, and notification.
U01_Q05_term	B	Afebrile means without fever.
U01_Q06_interaction	C	Active interaction requires the learner to apply judgment to the unit scenario.
U01_Q07_completion	D	The evidence model uses Moodle logs, attempts, submissions, and completion records.
U01_Q08_status	A	The course must not claim approval or issue certificates until approval status allows it.
U01_Q09_scenario	C	The first safe response is the scope-safe reporting/support action.
U01_Q10_guardrail	D	The course uses de-identified examples, online CE only, and no approval claim.

Moodle Activity Block - Assignment or Feedback: Reflection/Attestation

Prompt: Write the report you would give the licensed nurse using only fictional information.

Attestation checkbox: I completed this unit personally, used no real resident/facility information, understand this is online CE only, and understand certificate issuance remains blocked until all course and approval gates are satisfied.

Moodle Activity Block - Completion Gate: Required Activity Completion

Completion rule: U01 is complete only when the overview page is viewed, lesson content completion is met, scenario/application activity is completed, knowledge check is passed, reflection/attestation is submitted, and any configured minimum engagement/seat-time rule is satisfied.

Evidence generated by Moodle: page views, activity completion status, time/log records, H5P or lesson attempt data, quiz attempt and score, reflection/attestation submission, and unit completion report.

Instructor/admin notes: Review local policy terms before launch. For this unit, emphasize: measure as assigned, report promptly, and document factual observations.

Compliance guardrails: diagnose infection or decide medication needs is not allowed. Do not claim CDPH approval until issued. Do not count clinical or simulation hours. Do not copy government forms or signer binders into the course. Do not use old ContentV2 M00-M07 or an 8 x 3 structure as current.

U02 - Vital Signs II: Reporting and Abnormal Findings

Status: Draft / Pending CDPH Approval **Source backbone mapping:** Module 10 - Vital Signs (Objectives 5-16: pulse, respirations, abnormal breathing, combined TPR, blood pressure, equipment concepts, pain, charting, and reporting.)

Seat-time target: exactly 2 online CE hours (120 minutes). Suggested allocation: overview 10, lesson content 45, scenario/application 25, active interaction 15, knowledge check 15, reflection/attestation 10.

Moodle Activity Block - Page: Unit Overview

Purpose: This unit converts Module 10 - Vital Signs into learner-facing online CE content for CNA renewal support. It is online continuing education only and does not provide clinical hours, skills validation, or approval claims.

Learning objectives:

- ? Describe pulse, respiration, and blood-pressure observations using accurate terms.
- ? Identify reportable findings such as irregular pulse, labored breathing, cyanosis, dizziness, and unusual blood pressure.
- ? Use objective language when reporting abnormal vital-sign findings.
- ? Select safe escalation steps when a resident shows distress.

Compliance reminder: Use only fictional information in activities. Do not enter real resident, patient, facility, coworker, or family information. The course remains Draft / Pending CDPH Approval.

Moodle Activity Block - Page: Lesson Content

Pulse observations

Pulse observations include rate, rhythm, and strength. Terms such as radial, apical, bradycardia, tachycardia, arrhythmia, bounding, and thready help describe what is found. New irregularity or a pulse change with dizziness, chest discomfort, shortness of breath, or weakness should be reported promptly.

Respiration observations

Respirations include rate, rhythm, depth, effort, and sound. Dyspnea, cyanosis, apnea, tachypnea, bradypnea, shallow breathing, labored breathing, and Cheyne-Stokes respirations are reportable language. The CNA reports what is seen and heard; the nurse evaluates the cause.

Blood-pressure concepts

Blood pressure includes systolic pressure when the heart contracts and diastolic pressure when it relaxes. Readings may be affected by activity, pain, anxiety, cuff size, position, and timing. The CNA reports the reading, site, position if required, symptoms, and notification.

Abnormal findings and urgency

When a resident appears unstable, reporting comes before routine documentation. A strong report includes who, what, when, measured values, related observations, and current resident statement. Vague labels such as 'breathing bad' are replaced with facts such as 'respirations 26, labored, lips bluish.'

Builder note: Place this lesson content in a Moodle Page or Book page with completion tracking enabled for view plus minimum expected engagement time. Keep source-module names in the builder notes and do not brand or reproduce original CCCC/NATP source pages as learner pages.

Moodle Activity Block - H5P or Lesson Activity: Scenario/Application

Scenario: Resident B returns from the bathroom with pulse 112, respirations 26 and labored, and says, 'I cannot catch my breath.' Lips appear slightly bluish.

Required learner task: Branching lesson: choose urgent report wording, documentation language, and unsafe delayed responses.

Feedback design: Each incorrect path should explain the CNA scope issue, reporting delay, privacy risk, or documentation problem. Correct paths should reinforce observe, assist, report to the licensed nurse, document within scope, and protect dignity.

Moodle Activity Block - Quiz: Knowledge Check

Settings: 10 auto-graded multiple-choice items, shuffled answers, unlimited formative attempts or local policy limit, passing score 80 percent, feedback shown after attempt. Use 03_CNA_QUESTION_BANK_MOODLE_IMPORT_READY.csv for import.

Internal answer/rationale preview:

Question	Correct	Rationale
U02_Q01_source	A	U02 is mapped to Module 10 - Vital Signs within the 12 unit x 2 hour CNA course.
U02_Q02_priority	B	The CNA observes, reports, documents, and follows policy within scope.
U02_Q03_scope	C	The listed unsafe action exceeds CNA role or violates course guardrails.
U02_Q04_charting	D	Objective documentation records facts, statements, observations, and notification.
U02_Q05_term	B	Dyspnea means difficult or labored breathing.
U02_Q06_interaction	C	Active interaction requires the learner to apply judgment to the unit scenario.
U02_Q07_completion	D	The evidence model uses Moodle logs, attempts, submissions, and completion records.
U02_Q08_status	A	The course must not claim approval or issue certificates until approval status allows it.
U02_Q09_scenario	C	The first safe response is the scope-safe reporting/support action.

Question	Correct	Rationale
U02_Q10_guardrail	D	The course uses de-identified examples, online CE only, and no approval claim.

Moodle Activity Block - Assignment or Feedback: Reflection/Attestation

Prompt: Draft a one-sentence urgent report and one objective chart note.

Attestation checkbox: I completed this unit personally, used no real resident/facility information, understand this is online CE only, and understand certificate issuance remains blocked until all course and approval gates are satisfied.

Moodle Activity Block - Completion Gate: Required Activity Completion

Completion rule: U02 is complete only when the overview page is viewed, lesson content completion is met, scenario/application activity is completed, knowledge check is passed, reflection/attestation is submitted, and any configured minimum engagement/seat-time rule is satisfied.

Evidence generated by Moodle: page views, activity completion status, time/log records, H5P or lesson attempt data, quiz attempt and score, reflection/attestation submission, and unit completion report.

Instructor/admin notes: Review local policy terms before launch. For this unit, emphasize: stay with the resident as safe, notify the nurse/emergency chain, and report measured facts.

Compliance guardrails: wait until end of shift to chart because the resident walked recently is not allowed. Do not claim CDPH approval until issued. Do not count clinical or simulation hours. Do not copy government forms or signer binders into the course. Do not use old ContentV2 M00-M07 or an 8 x 3 structure as current.

U03 - Nutrition and Hydration

Status: Draft / Pending CDPH Approval **Source backbone mapping:** Module 11 - Nutrition (Objectives 1-10: terminology, food and fluids, nutrients, food guidance, elderly nutrition needs, therapeutic diets, feeding techniques, cultural/religious influences, and alternative nutrition.) **Seat-time target:** exactly 2 online CE hours (120 minutes). Suggested allocation: overview 10, lesson content 45, scenario/application 25, active interaction 15, knowledge check 15, reflection/attestation 10.

Moodle Activity Block - Page: Unit Overview

Purpose: This unit converts Module 11 - Nutrition into learner-facing online CE content for CNA renewal support. It is online continuing education only and does not provide clinical hours, skills validation, or approval claims.

Learning objectives:

- ? Describe the body's need for nutrients and fluids at a CNA observation level.
- ? Identify common therapeutic diets and the CNA responsibility to follow the ordered diet.
- ? Apply safe feeding-assistance principles that reduce aspiration risk and respect dignity.
- ? Recognize poor intake, dehydration, dysphagia, and diet-order concerns that require reporting.

Compliance reminder: Use only fictional information in activities. Do not enter real resident, patient, facility, coworker, or family information. The course remains Draft / Pending CDPH Approval.

Moodle Activity Block - Page: Lesson Content

Food and fluid needs

Nutrition provides energy, protein for tissue repair, vitamins and minerals for body function, fiber for bowel health, and fluid for circulation, temperature control, and elimination. Older adults may have reduced thirst, poor dentition, swallowing

difficulty, medication effects, limited appetite, or disease-related diet restrictions.

Therapeutic diets

Therapeutic diets support specific health needs. Examples include low sodium, low fat, diabetic or consistent carbohydrate, mechanical soft, pureed, thickened liquids, clear liquid, full liquid, and fluid restriction. The CNA follows the current diet order and stops to ask before giving food or fluid that does not match the plan.

Safe feeding assistance

Safe assistance begins with identification, hand hygiene, positioning, dentures or adaptive items, tray check, and resident readiness. The CNA offers small bites, allows time to chew and swallow, watches for coughing, wet voice, pocketing, drooling, or shortness of breath, and never forces feeding.

Intake, output, and reporting

Intake and output records help the nurse evaluate hydration and treatment response. CNAs document assigned meal percentages, milliliters, urine output, emesis, or other output accurately. Report repeated refusal, vomiting, diarrhea, edema, dehydration signs, aspiration signs, or concerns with feeding tubes or IV nutrition lines.

Builder note: Place this lesson content in a Moodle Page or Book page with completion tracking enabled for view plus minimum expected engagement time. Keep source-module names in the builder notes and do not brand or reproduce original CCCCO/NATP source pages as learner pages.

Moodle Activity Block - H5P or Lesson Activity: Scenario/Application

Scenario: Resident C is on nectar-thick liquids and a mechanical soft diet. A visitor leaves regular water and chips at the bedside. At lunch, Resident C coughs twice and has a wet-sounding voice.

Required learner task: H5P sequence: tray check, position, identify unsafe items, respond to cough/wet voice, and communicate respectfully.

Feedback design: Each incorrect path should explain the CNA scope issue, reporting delay, privacy risk, or documentation problem. Correct paths should reinforce observe, assist, report to the licensed nurse, document within scope, and protect dignity.

Moodle Activity Block - Quiz: Knowledge Check

Settings: 10 auto-graded multiple-choice items, shuffled answers, unlimited formative attempts or local policy limit, passing score 80 percent, feedback shown after attempt. Use 03_CNA_QUESTION_BANK_MOODLE_IMPORT_READY.csv for import.

Internal answer/rationale preview:

Question	Correct	Rationale
U03_Q01_source	A	U03 is mapped to Module 11 - Nutrition within the 12 unit x 2 hour CNA course.
U03_Q02_priority	B	The CNA observes, reports, documents, and follows policy within scope.
U03_Q03_scope	C	The listed unsafe action exceeds CNA role or violates course guardrails.
U03_Q04_charting	D	Objective documentation records facts, statements, observations, and notification.
U03_Q05_term	B	Dysphagia means difficulty swallowing.
U03_Q06_interaction	C	Active interaction requires the learner to apply judgment to the unit scenario.

Question	Correct	Rationale
U03_Q07_completion	D	The evidence model uses Moodle logs, attempts, submissions, and completion records.
U03_Q08_status	A	The course must not claim approval or issue certificates until approval status allows it.
U03_Q09_scenario	C	The first safe response is the scope-safe reporting/support action.
U03_Q10_guardrail	D	The course uses de-identified examples, online CE only, and no approval claim.

Moodle Activity Block - Assignment or Feedback: Reflection/Attestation

Prompt: Write how you would respond to the visitor while protecting dignity and following the care plan.

Attestation checkbox: I completed this unit personally, used no real resident/facility information, understand this is online CE only, and understand certificate issuance remains blocked until all course and approval gates are satisfied.

Moodle Activity Block - Completion Gate: Required Activity Completion

Completion rule: U03 is complete only when the overview page is viewed, lesson content completion is met, scenario/application activity is completed, knowledge check is passed, reflection/attestation is submitted, and any configured minimum engagement/seat-time rule is satisfied.

Evidence generated by Moodle: page views, activity completion status, time/log records, H5P or lesson attempt data, quiz attempt and score, reflection/attestation submission, and unit completion report.

Instructor/admin notes: Review local policy terms before launch. For this unit, emphasize: pause feeding as appropriate, keep the resident safe, report signs, and follow the ordered diet.

Compliance guardrails: change the diet order or offer thin water because the resident asks is not allowed. Do not claim CDPH approval until issued. Do not count clinical or simulation hours. Do not copy government forms or signer binders into the course. Do not use old ContentV2 M00-M07 or an 8 x 3 structure as current.

U04 - Emergency Procedures

Status: Draft / Pending CDPH Approval **Source backbone mapping:** Module 12 - Emergency Procedures (Objectives 1-5: emergency terminology, resident distress, immediate interventions, choking response, and common facility emergency codes.) **Seat-time target:** exactly 2 online CE hours (120 minutes). Suggested allocation: overview 10, lesson content 45, scenario/application 25, active interaction 15, knowledge check 15, reflection/attestation 10.

Moodle Activity Block - Page: Unit Overview

Purpose: This unit converts Module 12 - Emergency Procedures into learner-facing online CE content for CNA renewal support. It is online continuing education only and does not provide clinical hours, skills validation, or approval claims.

Learning objectives:

- ? Recognize signs and symptoms of resident distress that require immediate reporting.
- ? Describe the CNA role in activating help and following facility emergency procedures.
- ? Identify signs of choking and response boundaries at the cognitive CE level.
- ? Explain why online CE does not replace hands-on CPR, first-aid, or facility drill validation.

Compliance reminder: Use only fictional information in activities. Do not enter real resident, patient, facility, coworker, or family information. The course remains Draft / Pending CDPH Approval.

Moodle Activity Block - Page: Lesson Content

Emergency mindset

Emergencies require calm recognition, fast communication, and action within role. CNAs may be first to see chest pain, respiratory distress, choking, seizure activity, bleeding, fainting, sudden weakness, or change in consciousness. The first responsibility is to get help and protect the resident from immediate harm within facility policy.

Signs of distress

Urgent signs include crushing chest pain, pain radiating to arm, jaw, or back, severe shortness of breath, cyanosis, sudden confusion, facial droop, one-sided weakness, seizure, uncontrolled bleeding, syncope, shock signs, or hypoglycemia symptoms. Reports should be direct and factual.

Choking and airway concerns

Choking may appear as inability to speak, ineffective coughing, clutching the throat, cyanosis, panic, or collapse. Facility policy and current hands-on training determine response steps. This online unit teaches recognition and boundaries, not skill certification.

Codes, DNR, and team response

Facilities use emergency codes for events such as fire, medical emergency, missing resident, weather, or security. CNAs must know local code meanings, alarm locations, routes, and notification process. Advance directives and DNR orders are handled through licensed staff and facility procedure.

Builder note: Place this lesson content in a Moodle Page or Book page with completion tracking enabled for view plus minimum expected engagement time. Keep source-module names in the builder notes and do not brand or reproduce original CCCCO/NATP source pages as learner pages.

Moodle Activity Block - H5P or Lesson Activity: Scenario/Application

Scenario: Resident D suddenly stops speaking during dinner, grasps the throat, and cannot cough effectively. The dining room is noisy and one staff member says, 'Maybe it will pass.'

Required learner task: Branching lesson: choose immediate escalation, resident safety steps, post-event reporting, and documentation.

Feedback design: Each incorrect path should explain the CNA scope issue, reporting delay, privacy risk, or documentation problem. Correct paths should reinforce observe, assist, report to the licensed nurse, document within scope, and protect dignity.

Moodle Activity Block - Quiz: Knowledge Check

Settings: 10 auto-graded multiple-choice items, shuffled answers, unlimited formative attempts or local policy limit, passing score 80 percent, feedback shown after attempt. Use 03_CNA_QUESTION_BANK_MOODLE_IMPORT_READY.csv for import.

Internal answer/rationale preview:

Question	Correct	Rationale
U04_Q01_source	A	U04 is mapped to Module 12 - Emergency Procedures within the 12 unit x 2 hour CNA course.
U04_Q02_priority	B	The CNA observes, reports, documents, and follows policy within scope.
U04_Q03_scope	C	The listed unsafe action exceeds CNA role or violates course guardrails.

Question	Correct	Rationale
U04_Q04_charting	D	Objective documentation records facts, statements, observations, and notification.
U04_Q05_term	B	Cyanosis means bluish discoloration that can signal low oxygen.
U04_Q06_interaction	C	Active interaction requires the learner to apply judgment to the unit scenario.
U04_Q07_completion	D	The evidence model uses Moodle logs, attempts, submissions, and completion records.
U04_Q08_status	A	The course must not claim approval or issue certificates until approval status allows it.
U04_Q09_scenario	C	The first safe response is the scope-safe reporting/support action.
U04_Q10_guardrail	D	The course uses de-identified examples, online CE only, and no approval claim.

Moodle Activity Block - Assignment or Feedback: Reflection/Attestation

Prompt: Name two facts to report immediately and one action you would take only if trained and authorized.

Attestation checkbox: I completed this unit personally, used no real resident/facility information, understand this is online CE only, and understand certificate issuance remains blocked until all course and approval gates are satisfied.

Moodle Activity Block - Completion Gate: Required Activity Completion

Completion rule: U04 is complete only when the overview page is viewed, lesson content completion is met, scenario/application activity is completed, knowledge check is passed, reflection/attestation is submitted, and any configured minimum engagement/seat-time rule is satisfied.

Evidence generated by Moodle: page views, activity completion status, time/log records, H5P or lesson attempt data, quiz attempt and score, reflection/attestation submission, and unit completion report.

Instructor/admin notes: Review local policy terms before launch. For this unit, emphasize: activate help immediately and follow facility emergency procedure.

Compliance guardrails: wait quietly to see whether a complete airway obstruction resolves is not allowed. Do not claim CDPH approval until issued. Do not count clinical or simulation hours. Do not copy government forms or signer binders into the course. Do not use old ContentV2 M00-M07 or an 8 x 3 structure as current.

U05 - Long Term Care Patient/Resident I: Resident-Centered Care

Status: Draft / Pending CDPH Approval **Source backbone mapping:** Module 13 - Long Term Care Patient/Resident (Objectives 1-3 and 6-8: terminology, basic human needs, community resources, body organization, body systems, aging changes, and complications of immobility.) **Seat-time target:** exactly 2 online CE hours (120 minutes). Suggested allocation: overview 10, lesson content 45, scenario/application 25, active interaction 15, knowledge check 15, reflection/attestation 10.

Moodle Activity Block - Page: Unit Overview

Purpose: This unit converts Module 13 - Long Term Care Patient/Resident into learner-facing online CE content for CNA renewal support. It is online continuing education only and does not provide clinical hours, skills validation, or approval claims.

Learning objectives:

- ? Describe resident-centered care as support for physical, psychological, social, recreational, and spiritual needs.
- ? Summarize how aging can affect body systems without dismissing new changes as normal.
- ? Recognize observations related to skin, mobility, elimination, circulation, breathing, cognition, and appetite that require reporting.
- ? Use respectful person-first language.

Compliance reminder: Use only fictional information in activities. Do not enter real resident, patient, facility, coworker, or family information. The course remains Draft / Pending CDPH Approval.

Moodle Activity Block - Page: Lesson Content

Resident-centered care

Long-term care residents are adults with histories, preferences, routines, losses, strengths, and rights. A bath, transfer, meal, or toileting task is also a chance to preserve choice, privacy, dignity, and independence while following the care plan.

Aging and body systems

Aging can affect vision, hearing, skin, muscle strength, balance, digestion, bladder function, circulation, and memory. These changes do not mean pain, confusion, shortness of breath, pressure injury, falls, dehydration, or infection signs are normal or should be ignored.

Common conditions and observation

Residents may live with diabetes, COPD, CHF, arthritis, Parkinson's disease, dementia, stroke effects, pressure injury risk, urinary concerns, or sensory impairment. The CNA supports daily care and watches for changes rather than managing the diagnosis.

Quality of life and resources

Activities, social services, therapy, spiritual care, family, resident councils, ombudsman resources, and community programs can support quality of life. Reporting loneliness, withdrawal, grief, or unmet preferences can help the care team respond.

Builder note: Place this lesson content in a Moodle Page or Book page with completion tracking enabled for view plus minimum expected engagement time. Keep source-module names in the builder notes and do not brand or reproduce original CCCCO/NATP source pages as learner pages.

Moodle Activity Block - H5P or Lesson Activity: Scenario/Application

Scenario: Resident E used to attend morning activities but has stayed in bed most mornings this week. You notice less appetite, slower movement, and a new reddened area over the tailbone.

Required learner task: H5P sorting: resident preference, physical observation, psychosocial observation, reportable skin concern, and diagnosis.

Feedback design: Each incorrect path should explain the CNA scope issue, reporting delay, privacy risk, or documentation problem. Correct paths should reinforce observe, assist, report to the licensed nurse, document within scope, and protect dignity.

Moodle Activity Block - Quiz: Knowledge Check

Settings: 10 auto-graded multiple-choice items, shuffled answers, unlimited formative attempts or local policy limit, passing score 80 percent, feedback shown after attempt. Use 03_CNA_QUESTION_BANK_MOODLE_IMPORT_READY.csv for import.

Internal answer/rationale preview:

Question	Correct	Rationale
U05_Q01_source	A	U05 is mapped to Module 13 - Long Term Care Patient/Resident within the 12 unit x 2 hour CNA course.
U05_Q02_priority	B	The CNA observes, reports, documents, and follows policy within scope.
U05_Q03_scope	C	The listed unsafe action exceeds CNA role or violates course guardrails.
U05_Q04_charting	D	Objective documentation records facts, statements, observations, and notification.
U05_Q05_term	B	Baseline means the resident's usual condition or pattern used for comparison.
U05_Q06_interaction	C	Active interaction requires the learner to apply judgment to the unit scenario.
U05_Q07_completion	D	The evidence model uses Moodle logs, attempts, submissions, and completion records.
U05_Q08_status	A	The course must not claim approval or issue certificates until approval status allows it.
U05_Q09_scenario	C	The first safe response is the scope-safe reporting/support action.
U05_Q10_guardrail	D	The course uses de-identified examples, online CE only, and no approval claim.

Moodle Activity Block - Assignment or Feedback: Reflection/Attestation

Prompt: Write a respectful report that includes physical and psychosocial observations without labeling the resident.

Attestation checkbox: I completed this unit personally, used no real resident/facility information, understand this is online CE only, and understand certificate issuance remains blocked until all course and approval gates are satisfied.

Moodle Activity Block - Completion Gate: Required Activity Completion

Completion rule: U05 is complete only when the overview page is viewed, lesson content completion is met, scenario/application activity is completed, knowledge check is passed, reflection/attestation is submitted, and any configured minimum engagement/seat-time rule is satisfied.

Evidence generated by Moodle: page views, activity completion status, time/log records, H5P or lesson attempt data, quiz attempt and score, reflection/attestation submission, and unit completion report.

Instructor/admin notes: Review local policy terms before launch. For this unit, emphasize: compare to baseline, report objective changes, and continue care-plan support.

Compliance guardrails: dismiss changes as just old age or diagnose depression is not allowed. Do not claim CDPH approval until issued. Do not count clinical or simulation hours. Do not copy government forms or signer binders into the course. Do not use old ContentV2 M00-M07 or an 8 x 3 structure as current.

U06 - Long Term Care Patient/Resident II: Chronic Conditions and Daily Support

Status: Draft / Pending CDPH Approval **Source backbone mapping:** Module 13 - Long Term Care Patient/Resident (Objectives 2-5 and 7-8: human needs, community resources, developmental and mental disorders, Alzheimer's disease and related dementias, body systems, aging changes, and CNA observations.) **Seat-time target:** exactly 2 online CE hours (120 minutes). Suggested allocation: overview 10, lesson content 45, scenario/application 25, active interaction 15, knowledge check 15, reflection/attestation 10.

Moodle Activity Block - Page: Unit Overview

Purpose: This unit converts Module 13 - Long Term Care Patient/Resident into learner-facing online CE content for CNA renewal support. It is online continuing education only and does not provide clinical hours, skills validation, or approval claims.

Learning objectives:

- ? Apply person-centered approaches for residents with dementia, developmental disability, mental health needs, or chronic illness.
- ? Distinguish validation, redirection, reality orientation, and reminiscence as care-plan guided approaches.
- ? Identify daily-support observations for chronic conditions and report behavior or function changes without stigma.
- ? Describe how consistency, environment, and communication can reduce distress.

Compliance reminder: Use only fictional information in activities. Do not enter real resident, patient, facility, coworker, or family information. The course remains Draft / Pending CDPH Approval.

Moodle Activity Block - Page: Lesson Content

Chronic conditions and daily life

Chronic conditions affect how a resident eats, moves, breathes, communicates, sleeps, and participates in care. The CNA supports routines and watches for changes that may indicate worsening condition or unmet need while staying within the care plan.

Dementia-sensitive care

Dementia can affect memory, judgment, language, perception, and emotional regulation. Behavior is communication. Wandering, calling out, resistance, or repeated questions may reflect pain, fear, hunger, toileting need, fatigue, or an overwhelming environment.

Mental health and developmental needs

Residents may have intellectual disability, cerebral palsy, epilepsy, Parkinson's disease, anxiety, depression, bipolar disorder, schizophrenia, PTSD, or other conditions. Respectful care focuses on abilities, preferences, safety, and communication, not labels.

Supporting function and dignity

Daily support includes allowing time, offering choices, explaining before touch, protecting privacy, encouraging what the resident can do, and using adaptive equipment as directed. CNAs contribute valuable reports about baseline, changes, triggers, and successful approaches.

Builder note: Place this lesson content in a Moodle Page or Book page with completion tracking enabled for view plus minimum expected engagement time. Keep source-module names in the builder notes and do not brand or reproduce original CCCCO/NATP source pages as learner pages.

Moodle Activity Block - H5P or Lesson Activity: Scenario/Application

Scenario: Resident F, who has dementia, becomes upset during evening care and says, 'I need to go home to feed my children.' The resident pulls away when rushed but calms when shown a family photo.

Required learner task: Branching scenario: choose validation, redirection, safety support, and objective report wording.

Feedback design: Each incorrect path should explain the CNA scope issue, reporting delay, privacy risk, or documentation problem. Correct paths should reinforce observe, assist, report to the licensed nurse, document within scope, and protect dignity.

Moodle Activity Block - Quiz: Knowledge Check

Settings: 10 auto-graded multiple-choice items, shuffled answers, unlimited formative attempts or local policy limit, passing score 80 percent, feedback shown after attempt. Use 03_CNA_QUESTION_BANK_MOODLE_IMPORT_READY.csv for import.

Internal answer/rationale preview:

Question	Correct	Rationale
U06_Q01_source	A	U06 is mapped to Module 13 - Long Term Care Patient/Resident within the 12 unit x 2 hour CNA course.
U06_Q02_priority	B	The CNA observes, reports, documents, and follows policy within scope.
U06_Q03_scope	C	The listed unsafe action exceeds CNA role or violates course guardrails.
U06_Q04_charting	D	Objective documentation records facts, statements, observations, and notification.
U06_Q05_term	B	Validation therapy means acknowledging the resident's feelings or perceived reality to reduce distress.
U06_Q06_interaction	C	Active interaction requires the learner to apply judgment to the unit scenario.
U06_Q07_completion	D	The evidence model uses Moodle logs, attempts, submissions, and completion records.
U06_Q08_status	A	The course must not claim approval or issue certificates until approval status allows it.
U06_Q09_scenario	C	The first safe response is the scope-safe reporting/support action.
U06_Q10_guardrail	D	The course uses de-identified examples, online CE only, and no approval claim.

Moodle Activity Block - Assignment or Feedback: Reflection/Attestation

Prompt: Describe two supportive responses and one objective observation to report.

Attestation checkbox: I completed this unit personally, used no real resident/facility information, understand this is online CE only, and understand certificate issuance remains blocked until all course and approval gates are satisfied.

Moodle Activity Block - Completion Gate: Required Activity Completion

Completion rule: U06 is complete only when the overview page is viewed, lesson content completion is met, scenario/application activity is completed, knowledge check is passed, reflection/attestation is submitted, and any configured minimum engagement/seat-time rule is satisfied.

Evidence generated by Moodle: page views, activity completion status, time/log records, H5P or lesson attempt data, quiz attempt and score, reflection/attestation submission, and unit completion report.

Instructor/admin notes: Review local policy terms before launch. For this unit, emphasize: use care-plan guided communication, reduce triggers, and report objective patterns.

Compliance guardrails: argue, shame, force care, or label the resident as bad is not allowed. Do not claim CDPH approval until issued. Do not count clinical or simulation hours. Do not copy government forms or signer binders into the course. Do not use old ContentV2 M00-M07 or an 8 x 3 structure as current.

U07 - Rehabilitative Nursing

Status: Draft / Pending CDPH Approval **Source backbone mapping:** Module 14 - Rehabilitative Nursing (Objectives 1-12: restorative philosophy, rehab team, CNA responsibilities, ADLs, devices, inactivity complications, ROM, mobility, self-esteem, family involvement, documentation, and care-plan meetings.) **Seat-time target:** exactly 2 online CE hours (120 minutes). Suggested allocation: overview 10, lesson content 45, scenario/application 25, active interaction 15, knowledge check 15, reflection/attestation 10.

Moodle Activity Block - Page: Unit Overview

Purpose: This unit converts Module 14 - Rehabilitative Nursing into learner-facing online CE content for CNA renewal support. It is online continuing education only and does not provide clinical hours, skills validation, or approval claims.

Learning objectives:

- ? Explain restorative care as support for the resident's highest practical level of function.
- ? Describe the CNA role as a member of the rehabilitation/restorative team.
- ? Promote ADL participation and self-care without taking over tasks the resident can safely do.
- ? Recognize complications of inactivity and document restorative observations as assigned.

Compliance reminder: Use only fictional information in activities. Do not enter real resident, patient, facility, coworker, or family information. The course remains Draft / Pending CDPH Approval.

Moodle Activity Block - Page: Lesson Content

Restorative philosophy

Rehabilitative nursing and restorative care focus on maintaining or improving function. The goal is not speed for staff; the goal is helping the resident do as much as safely possible. Independence may mean brushing part of the hair, taking three steps with assistance, or using an adaptive utensil.

Team roles and care plan

The interdisciplinary team may include nursing, therapy, activities, social services, dietary, physicians, family, and the resident. The CNA follows the care plan and reports what the resident can do, what help was needed, pain, fatigue, device concerns, refusal, and progress.

ADLs, devices, and mobility

ADLs include bathing, dressing, grooming, eating, toileting, mobility, and transferring. Adaptive and comfort devices may include walkers, canes, wheelchairs, splints, prostheses, cushions, heel protectors, foot boards, bed cradles, or special mattresses. CNAs watch for skin, pain, fit, and safety concerns.

Preventing inactivity complications

Immobility can contribute to pressure injuries, contractures, pneumonia, constipation, weakness, thrombophlebitis, edema, and loss of confidence. Range of motion may be active, passive, or active-assistive; CNAs perform only assigned activities for which they are trained and validated.

Builder note: Place this lesson content in a Moodle Page or Book page with completion tracking enabled for view plus minimum expected engagement time. Keep source-module names in the builder notes and do not brand or reproduce original CCCCO/NATP source pages as learner pages.

Moodle Activity Block - H5P or Lesson Activity: Scenario/Application

Scenario: Resident G can wash the face and upper body with setup but asks you to do everything because it is faster. The care plan encourages self-care and use of a long-handled sponge.

Required learner task: H5P choice activity: select phrases and setup actions that encourage independence while respecting fatigue and dignity.

Feedback design: Each incorrect path should explain the CNA scope issue, reporting delay, privacy risk, or documentation problem. Correct paths should reinforce observe, assist, report to the licensed nurse, document within scope, and protect dignity.

Moodle Activity Block - Quiz: Knowledge Check

Settings: 10 auto-graded multiple-choice items, shuffled answers, unlimited formative attempts or local policy limit, passing score 80 percent, feedback shown after attempt. Use 03_CNA_QUESTION_BANK_MOODLE_IMPORT_READY.csv for import.

Internal answer/rationale preview:

Question	Correct	Rationale
U07_Q01_source	A	U07 is mapped to Module 14 - Rehabilitative Nursing within the 12 unit x 2 hour CNA course.
U07_Q02_priority	B	The CNA observes, reports, documents, and follows policy within scope.
U07_Q03_scope	C	The listed unsafe action exceeds CNA role or violates course guardrails.
U07_Q04_charting	D	Objective documentation records facts, statements, observations, and notification.
U07_Q05_term	B	Restorative care means care that supports the resident's highest practical level of function.
U07_Q06_interaction	C	Active interaction requires the learner to apply judgment to the unit scenario.
U07_Q07_completion	D	The evidence model uses Moodle logs, attempts, submissions, and completion records.
U07_Q08_status	A	The course must not claim approval or issue certificates until approval status allows it.
U07_Q09_scenario	C	The first safe response is the scope-safe reporting/support action.
U07_Q10_guardrail	D	The course uses de-identified examples, online CE only, and no approval claim.

Moodle Activity Block - Assignment or Feedback: Reflection/Attestation

Prompt: Write how you would support self-care and what you would document.

Attestation checkbox: I completed this unit personally, used no real resident/facility information, understand this is online CE only, and understand certificate issuance remains blocked until all course and approval gates are satisfied.

Moodle Activity Block - Completion Gate: Required Activity Completion

Completion rule: U07 is complete only when the overview page is viewed, lesson content completion is met, scenario/application activity is completed, knowledge check is passed, reflection/attestation is submitted, and any configured minimum engagement/seat-time rule is satisfied.

Evidence generated by Moodle: page views, activity completion status, time/log records, H5P or lesson attempt data, quiz attempt and score, reflection/attestation submission, and unit completion report.

Instructor/admin notes: Review local policy terms before launch. For this unit, emphasize: encourage safe participation, follow the care plan, and report functional changes.

Compliance guardrails: change the mobility or therapy plan independently is not allowed. Do not claim CDPH approval until issued. Do not count clinical or simulation hours. Do not copy government forms or signer binders into the course. Do not use old ContentV2 M00-M07 or an 8 x 3 structure as current.

U08 - Observation and Charting I: Objective Observation

Status: Draft / Pending CDPH Approval **Source backbone mapping:** Module 15 - Observation and Charting (Objectives 1-5: terminology, word elements, abbreviations, observation, senses used to observe, and objective versus subjective observations.) **Seat-time target:** exactly 2 online CE hours (120 minutes). Suggested allocation: overview 10, lesson content 45, scenario/application 25, active interaction 15, knowledge check 15, reflection/attestation 10.

Moodle Activity Block - Page: Unit Overview

Purpose: This unit converts Module 15 - Observation and Charting into learner-facing online CE content for CNA renewal support. It is online continuing education only and does not provide clinical hours, skills validation, or approval claims.

Learning objectives:

- ? Define observation and identify senses used to collect CNA observations.
- ? Distinguish objective observations from subjective statements.
- ? Use approved terminology and abbreviations only as allowed by facility policy.
- ? Convert vague wording into factual observation language.

Compliance reminder: Use only fictional information in activities. Do not enter real resident, patient, facility, coworker, or family information. The course remains Draft / Pending CDPH Approval.

Moodle Activity Block - Page: Lesson Content

Observation as active skill

Observation is intentional noticing. CNAs observe appearance, behavior, mobility, speech, breathing, skin, appetite, elimination, pain statements, mood, and safety during routine care. Good observation compares what is happening now with the resident's usual pattern.

Objective and subjective

Objective information is seen, heard, smelled, measured, or directly observed. Subjective information is what the resident reports or what another person says. Both can matter, but they should be labeled clearly and reported accurately.

Terminology and abbreviations

Medical words often use roots, prefixes, suffixes, and combining vowels. Abbreviations can create errors if misunderstood. CNAs should use only facility-approved abbreviations and ask when unsure.

Reporting cues

Report new pain, falls, dizziness, shortness of breath, confusion, meal refusal, vomiting, diarrhea, constipation, skin redness, swelling, bleeding, mood change, unsafe behavior, or ADL change. Strong reports answer who, what, when, where, what was observed, what the resident said, and who was notified.

Builder note: Place this lesson content in a Moodle Page or Book page with completion tracking enabled for view plus minimum expected engagement time. Keep source-module names in the builder notes and do not brand or reproduce original CCCCO/NATP source pages as learner pages.

Moodle Activity Block - H5P or Lesson Activity: Scenario/Application

Scenario: Resident H says, 'I feel dizzy,' grips the bedrail while standing, appears pale, and ate only 10 percent of breakfast.

Required learner task: H5P table: label items as objective, subjective, measurement, or interpretation, then rewrite vague notes.

Feedback design: Each incorrect path should explain the CNA scope issue, reporting delay, privacy risk, or documentation problem. Correct paths should reinforce observe, assist, report to the licensed nurse, document within scope, and protect dignity.

Moodle Activity Block - Quiz: Knowledge Check

Settings: 10 auto-graded multiple-choice items, shuffled answers, unlimited formative attempts or local policy limit, passing score 80 percent, feedback shown after attempt. Use 03_CNA_QUESTION_BANK_MOODLE_IMPORT_READY.csv for import.

Internal answer/rationale preview:

Question	Correct	Rationale
U08_Q01_source	A	U08 is mapped to Module 15 - Observation and Charting within the 12 unit x 2 hour CNA course.
U08_Q02_priority	B	The CNA observes, reports, documents, and follows policy within scope.
U08_Q03_scope	C	The listed unsafe action exceeds CNA role or violates course guardrails.
U08_Q04_charting	D	Objective documentation records facts, statements, observations, and notification.
U08_Q05_term	B	Objective observation means information seen, heard, smelled, measured, or directly observed.
U08_Q06_interaction	C	Active interaction requires the learner to apply judgment to the unit scenario.
U08_Q07_completion	D	The evidence model uses Moodle logs, attempts, submissions, and completion records.
U08_Q08_status	A	The course must not claim approval or issue certificates until approval status allows it.
U08_Q09_scenario	C	The first safe response is the scope-safe reporting/support action.
U08_Q10_guardrail	D	The course uses de-identified examples, online CE only, and no approval claim.

Moodle Activity Block - Assignment or Feedback: Reflection/Attestation

Prompt: Rewrite this objectively: 'Resident was acting weird and probably had low blood sugar.'

Attestation checkbox: I completed this unit personally, used no real resident/facility information, understand this is online CE only, and understand certificate issuance remains blocked until all course and approval gates are satisfied.

Moodle Activity Block - Completion Gate: Required Activity Completion

Completion rule: U08 is complete only when the overview page is viewed, lesson content completion is met, scenario/application activity is completed, knowledge check is passed, reflection/attestation is submitted, and any configured minimum engagement/seat-time rule is satisfied.

Evidence generated by Moodle: page views, activity completion status, time/log records, H5P or lesson attempt data, quiz attempt and score, reflection/attestation submission, and unit completion report.

Instructor/admin notes: Review local policy terms before launch. For this unit, emphasize: report objective and subjective details promptly using approved terms.

Compliance guardrails: diagnose the cause or use vague labels like acting weird is not allowed. Do not claim CDPH approval until issued. Do not count clinical or simulation hours. Do not copy government forms or signer binders into the course. Do not use old ContentV2 M00-M07 or an 8 x 3 structure as current.

U09 - Observation and Charting II: Documentation and Reporting

Status: Draft / Pending CDPH Approval **Source backbone mapping:** Module 15 - Observation and Charting (Objectives 6-8: charting documents, ADL assessment for MDS, and procedures for recording on the resident chart.) **Seat-time target:** exactly 2 online CE hours (120 minutes). Suggested allocation: overview 10, lesson content 45, scenario/application 25, active interaction 15, knowledge check 15, reflection/attestation 10.

Moodle Activity Block - Page: Unit Overview

Purpose: This unit converts Module 15 - Observation and Charting into learner-facing online CE content for CNA renewal support. It is online continuing education only and does not provide clinical hours, skills validation, or approval claims.

Learning objectives:

- ? Identify common charting documents and the CNA contribution to each.
- ? Document ADL assistance accurately for care planning and MDS-related records as assigned.
- ? Apply legal charting principles: timely, factual, complete, legible, and corrected according to policy.
- ? Protect confidentiality in electronic and paper documentation.

Compliance reminder: Use only fictional information in activities. Do not enter real resident, patient, facility, coworker, or family information. The course remains Draft / Pending CDPH Approval.

Moodle Activity Block - Page: Lesson Content

Why documentation matters

Documentation communicates resident status, supports continuity between shifts, contributes to care planning, and creates a legal record. If care was not charted, the team may not know it happened. If a change was charted but not reported, the resident may still be at risk.

ADL and MDS-related accuracy

ADL documentation should reflect what the resident actually did and how much assistance was provided. Overstating independence or dependence can distort care planning. CNAs should know facility definitions for setup help, supervision, limited assist, extensive assist, total dependence, and activity did not occur.

Legal charting principles

Good charting is timely, factual, objective, complete, and within scope. It uses approved abbreviations, avoids blame, and records corrections according to facility policy. Never erase, backdate, falsify, copy-forward inaccurately, or chart care before it is done.

Confidentiality and incident reporting

Resident information is confidential. CNAs protect screens, log out, avoid hallway conversations, and never enter real resident data into practice assignments. For urgent changes, notify the nurse first, then document according to policy.

Builder note: Place this lesson content in a Moodle Page or Book page with completion tracking enabled for view plus minimum expected engagement time. Keep source-module names in the builder notes and do not brand or reproduce original CCCCO/NATP source pages as learner pages.

Moodle Activity Block - H5P or Lesson Activity: Scenario/Application

Scenario: Resident I slid from the wheelchair to the floor while reaching for a sweater. The resident denies pain, but you see a small skin tear on the forearm. Another aide says, 'Just chart assisted with transfer.'

Required learner task: Branching documentation exercise: choose immediate report, incident facts, prohibited wording, and correction process.

Feedback design: Each incorrect path should explain the CNA scope issue, reporting delay, privacy risk, or documentation problem. Correct paths should reinforce observe, assist, report to the licensed nurse, document within scope, and protect dignity.

Moodle Activity Block - Quiz: Knowledge Check

Settings: 10 auto-graded multiple-choice items, shuffled answers, unlimited formative attempts or local policy limit, passing score 80 percent, feedback shown after attempt. Use 03_CNA_QUESTION_BANK_MOODLE_IMPORT_READY.csv for import.

Internal answer/rationale preview:

Question	Correct	Rationale
U09_Q01_source	A	U09 is mapped to Module 15 - Observation and Charting within the 12 unit x 2 hour CNA course.
U09_Q02_priority	B	The CNA observes, reports, documents, and follows policy within scope.
U09_Q03_scope	C	The listed unsafe action exceeds CNA role or violates course guardrails.
U09_Q04_charting	D	Objective documentation records facts, statements, observations, and notification.
U09_Q05_term	B	Incident report means a facility document used to record facts about an unusual event according to policy.
U09_Q06_interaction	C	Active interaction requires the learner to apply judgment to the unit scenario.
U09_Q07_completion	D	The evidence model uses Moodle logs, attempts, submissions, and completion records.
U09_Q08_status	A	The course must not claim approval or issue certificates until approval status allows it.
U09_Q09_scenario	C	The first safe response is the scope-safe reporting/support action.
U09_Q10_guardrail	D	The course uses de-identified examples, online CE only, and no approval claim.

Moodle Activity Block - Assignment or Feedback: Reflection/Attestation

Prompt: Write two objective documentation statements and identify who must be notified.

Attestation checkbox: I completed this unit personally, used no real resident/facility information, understand this is online CE only, and understand certificate issuance remains blocked until all course and approval gates are satisfied.

Moodle Activity Block - Completion Gate: Required Activity Completion

Completion rule: U09 is complete only when the overview page is viewed, lesson content completion is met, scenario/application activity is completed, knowledge check is passed, reflection/attestation is submitted, and any configured minimum engagement/seat-time rule is satisfied.

Evidence generated by Moodle: page views, activity completion status, time/log records, H5P or lesson attempt data, quiz attempt and score, reflection/attestation submission, and unit completion report.

Instructor/admin notes: Review local policy terms before launch. For this unit, emphasize: report immediately, document facts, and follow incident policy.

Compliance guardrails: chart a false assisted transfer to avoid blame is not allowed. Do not claim CDPH approval until issued. Do not count clinical or simulation hours. Do not copy government forms or signer binders into the course. Do not use old ContentV2 M00-M07 or an 8 x 3 structure as current.

U10 - Death and Dying

Status: Draft / Pending CDPH Approval **Source backbone mapping:** Module 16 - Death and Dying (Objectives 1-8: terminology, grief stages, emotional/spiritual needs, rights of the dying resident, signs of approaching and biological death, comfort measures, hospice, and postmortem care responsibilities.) **Seat-time target:** exactly 2 online CE hours (120 minutes). Suggested allocation: overview 10, lesson content 45, scenario/application 25, active interaction 15, knowledge check 15, reflection/attestation 10.

Moodle Activity Block - Page: Unit Overview

Purpose: This unit converts Module 16 - Death and Dying into learner-facing online CE content for CNA renewal support. It is online continuing education only and does not provide clinical hours, skills validation, or approval claims.

Learning objectives:

- ? Describe grief responses using sensitive, nonjudgmental language.
- ? Identify emotional, spiritual, cultural, and family-support needs at end of life.
- ? Recognize signs of approaching death and report changes within scope.
- ? Describe CNA comfort-care support under the care plan and licensed direction.

Compliance reminder: Use only fictional information in activities. Do not enter real resident, patient, facility, coworker, or family information. The course remains Draft / Pending CDPH Approval.

Moodle Activity Block - Page: Lesson Content

Understanding grief

Death and dying care asks the CNA to combine skill, calm, privacy, and compassion. Grief may include denial, anger, bargaining, depression, acceptance, or other responses. People do not move through grief in a fixed order, and families may respond differently from the resident.

Rights, culture, and spiritual needs

Dying residents retain the right to dignity, privacy, choice, comfort support, visitors according to policy, cultural and spiritual practices, and respectful care. The CNA should ask preferences when appropriate and refer spiritual or family concerns to the nurse, chaplain, or social worker.

Signs and comfort measures

Signs of approaching death may include decreased appetite, increased sleep, withdrawal, breathing changes, cool extremities, mottling, decreased urine output, restlessness, or altered consciousness. Comfort measures may include mouth care, repositioning, clean linens, quiet environment, gentle presence, and skin care.

Hospice and postmortem boundaries

Hospice focuses on comfort and quality of life. CNAs may provide personal care, observation, and family support alongside hospice and facility staff. Postmortem care follows facility policy, legal requirements, family/cultural wishes, and dignity protections.

Builder note: Place this lesson content in a Moodle Page or Book page with completion tracking enabled for view plus minimum expected engagement time. Keep source-module names in the builder notes and do not brand or reproduce original CCCCO/NATP source pages as learner pages.

Moodle Activity Block - H5P or Lesson Activity: Scenario/Application

Scenario: Resident J is receiving hospice services. The family asks if the resident is 'about to die' because breathing has long pauses. The resident's mouth appears dry.

Required learner task: H5P response selection: supportive words, reportable observations, comfort measures within scope, and questions to refer to the nurse.

Feedback design: Each incorrect path should explain the CNA scope issue, reporting delay, privacy risk, or documentation problem. Correct paths should reinforce observe, assist, report to the licensed nurse, document within scope, and protect dignity.

Moodle Activity Block - Quiz: Knowledge Check

Settings: 10 auto-graded multiple-choice items, shuffled answers, unlimited formative attempts or local policy limit, passing score 80 percent, feedback shown after attempt. Use 03_CNA_QUESTION_BANK_MOODLE_IMPORT_READY.csv for import.

Internal answer/rationale preview:

Question	Correct	Rationale
U10_Q01_source	A	U10 is mapped to Module 16 - Death and Dying within the 12 unit x 2 hour CNA course.
U10_Q02_priority	B	The CNA observes, reports, documents, and follows policy within scope.
U10_Q03_scope	C	The listed unsafe action exceeds CNA role or violates course guardrails.
U10_Q04_charting	D	Objective documentation records facts, statements, observations, and notification.
U10_Q05_term	B	Hospice means care focused on comfort and quality of life for a person with terminal illness.
U10_Q06_interaction	C	Active interaction requires the learner to apply judgment to the unit scenario.
U10_Q07_completion	D	The evidence model uses Moodle logs, attempts, submissions, and completion records.
U10_Q08_status	A	The course must not claim approval or issue certificates until approval status allows it.
U10_Q09_scenario	C	The first safe response is the scope-safe reporting/support action.

Question	Correct	Rationale
U10_Q10_guardrail	D	The course uses de-identified examples, online CE only, and no approval claim.

Moodle Activity Block - Assignment or Feedback: Reflection/Attestation

Prompt: Write a compassionate response to the family and a separate factual report to the nurse.

Attestation checkbox: I completed this unit personally, used no real resident/facility information, understand this is online CE only, and understand certificate issuance remains blocked until all course and approval gates are satisfied.

Moodle Activity Block - Completion Gate: Required Activity Completion

Completion rule: U10 is complete only when the overview page is viewed, lesson content completion is met, scenario/application activity is completed, knowledge check is passed, reflection/attestation is submitted, and any configured minimum engagement/seat-time rule is satisfied.

Evidence generated by Moodle: page views, activity completion status, time/log records, H5P or lesson attempt data, quiz attempt and score, reflection/attestation submission, and unit completion report.

Instructor/admin notes: Review local policy terms before launch. For this unit, emphasize: provide assigned comfort care, support privacy, report changes, and refer prognosis questions.

Compliance guardrails: predict exact time of death or interpret DNR orders independently is not allowed. Do not claim CDPH approval until issued. Do not count clinical or simulation hours. Do not copy government forms or signer binders into the course. Do not use old ContentV2 M00-M07 or an 8 x 3 structure as current.

U11 - Patient/Resident Abuse I: Recognition and Prevention

Status: Draft / Pending CDPH Approval **Source backbone mapping:** Module 17 - Patient/Resident Abuse (Objectives 1-4: terminology, types of elder abuse, issues related to elder abuse, and CNA role in prevention.) **Seat-time target:** exactly 2 online CE hours (120 minutes). Suggested allocation: overview 10, lesson content 45, scenario/application 25, active interaction 15, knowledge check 15, reflection/attestation 10.

Moodle Activity Block - Page: Unit Overview

Purpose: This unit converts Module 17 - Patient/Resident Abuse into learner-facing online CE content for CNA renewal support. It is online continuing education only and does not provide clinical hours, skills validation, or approval claims.

Learning objectives:

- ? Define abuse, neglect, financial abuse, psychological abuse, physical abuse, sexual abuse, verbal abuse, and involuntary seclusion.
- ? Recognize signs, patterns, and risk factors that may indicate abuse or neglect.
- ? Explain how resident rights, privacy, choice, and dignity reduce abuse risk.
- ? Distinguish prevention and recognition from investigation or accusation.

Compliance reminder: Use only fictional information in activities. Do not enter real resident, patient, facility, coworker, or family information. The course remains Draft / Pending CDPH Approval.

Moodle Activity Block - Page: Lesson Content

Rights and prevention

Abuse prevention begins with daily respect. Residents have rights to dignity, privacy, choice, informed participation, confidentiality, freedom from abuse or neglect, and voice in grievances. CNAs protect rights by knocking, explaining care,

honoring preferences when possible, and refusing humiliating or coercive behavior.

Types of abuse and neglect

Abuse may be physical, verbal, psychological, sexual, financial, neglect, abandonment, involuntary seclusion, or healthcare fraud. Neglect can be active or passive but still harms residents. Financial abuse can include theft, coercion, or misuse of funds or property.

Signs and patterns

Possible signs include unexplained bruises, burns, fearfulness, withdrawal, sudden behavior change, poor hygiene, dehydration, weight loss, missing belongings, untreated pain, restraint marks, or a resident statement that someone hurt or threatened them. One sign does not prove abuse, but every concern must be taken seriously.

Professional boundaries

Professional boundaries protect residents and staff. CNAs should not borrow money, accept improper gifts, share private information, use social media involving residents, threaten, mock, isolate, or retaliate. Frustration is a signal to get help, not a reason to use force or harsh words.

Builder note: Place this lesson content in a Moodle Page or Book page with completion tracking enabled for view plus minimum expected engagement time. Keep source-module names in the builder notes and do not brand or reproduce original CCCCO/NATP source pages as learner pages.

Moodle Activity Block - H5P or Lesson Activity: Scenario/Application

Scenario: Resident K flinches when a certain caregiver enters and says quietly, 'Please do not leave me with that person.' You also notice new bruising on the upper arm.

Required learner task: H5P classification: possible abuse signs, immediate safety actions, objective report language, and investigation beyond CNA role.

Feedback design: Each incorrect path should explain the CNA scope issue, reporting delay, privacy risk, or documentation problem. Correct paths should reinforce observe, assist, report to the licensed nurse, document within scope, and protect dignity.

Moodle Activity Block - Quiz: Knowledge Check

Settings: 10 auto-graded multiple-choice items, shuffled answers, unlimited formative attempts or local policy limit, passing score 80 percent, feedback shown after attempt. Use 03_CNA_QUESTION_BANK_MOODLE_IMPORT_READY.csv for import.

Internal answer/rationale preview:

Question	Correct	Rationale
U11_Q01_source	A	U11 is mapped to Module 17 - Patient/Resident Abuse within the 12 unit x 2 hour CNA course.
U11_Q02_priority	B	The CNA observes, reports, documents, and follows policy within scope.
U11_Q03_scope	C	The listed unsafe action exceeds CNA role or violates course guardrails.
U11_Q04_charting	D	Objective documentation records facts, statements, observations, and notification.
U11_Q05_term	B	Mandated reporter means a person legally required to report suspected abuse through required channels.

Question	Correct	Rationale
U11_Q06_interaction	C	Active interaction requires the learner to apply judgment to the unit scenario.
U11_Q07_completion	D	The evidence model uses Moodle logs, attempts, submissions, and completion records.
U11_Q08_status	A	The course must not claim approval or issue certificates until approval status allows it.
U11_Q09_scenario	C	The first safe response is the scope-safe reporting/support action.
U11_Q10_guardrail	D	The course uses de-identified examples, online CE only, and no approval claim.

Moodle Activity Block - Assignment or Feedback: Reflection/Attestation

Prompt: Write what you would report without accusing or investigating.

Attestation checkbox: I completed this unit personally, used no real resident/facility information, understand this is online CE only, and understand certificate issuance remains blocked until all course and approval gates are satisfied.

Moodle Activity Block - Completion Gate: Required Activity Completion

Completion rule: U11 is complete only when the overview page is viewed, lesson content completion is met, scenario/application activity is completed, knowledge check is passed, reflection/attestation is submitted, and any configured minimum engagement/seat-time rule is satisfied.

Evidence generated by Moodle: page views, activity completion status, time/log records, H5P or lesson attempt data, quiz attempt and score, reflection/attestation submission, and unit completion report.

Instructor/admin notes: Review local policy terms before launch. For this unit, emphasize: ensure immediate safety as directed, report concern promptly, and document objective facts.

Compliance guardrails: investigate privately, confront the suspected person, or stay silent is not allowed. Do not claim CDPH approval until issued. Do not count clinical or simulation hours. Do not copy government forms or signer binders into the course. Do not use old ContentV2 M00-M07 or an 8 x 3 structure as current.

U12 - Patient/Resident Abuse II: Reporting and Response

Status: Draft / Pending CDPH Approval **Source backbone mapping:** Module 17 - Patient/Resident Abuse (Objectives 1-5: terminology, abuse types, issues, prevention, and CNA role in reporting elder abuse.) **Seat-time target:** exactly 2 online CE hours (120 minutes). Suggested allocation: overview 10, lesson content 45, scenario/application 25, active interaction 15, knowledge check 15, reflection/attestation 10.

Moodle Activity Block - Page: Unit Overview

Purpose: This unit converts Module 17 - Patient/Resident Abuse into learner-facing online CE content for CNA renewal support. It is online continuing education only and does not provide clinical hours, skills validation, or approval claims.

Learning objectives:

- ? Describe mandated-reporter responsibilities at a high level and the need to follow current California/facility procedures.
- ? Select immediate response steps when abuse, neglect, exploitation, or rights violations are suspected.
- ? Document and report objectively while preserving confidentiality.
- ? Complete the final applied scenario and course attestation before certificate review.

Compliance reminder: Use only fictional information in activities. Do not enter real resident, patient, facility, coworker, or family information. The course remains Draft / Pending CDPH Approval.

Moodle Activity Block - Page: Lesson Content

Reporting duty and urgency

Patient/resident abuse reporting is not optional. CNAs must follow mandated-reporter law, facility policy, and supervisory chain. Because reporting requirements can change and may have strict timelines, the Moodle build must link to current approved California and facility reporting instructions.

Immediate response

Immediate response focuses on resident safety, prompt notification, confidentiality, and factual documentation. If a resident is in immediate danger, follow emergency and supervisor notification procedures. Do not confront the suspected abuser in a way that increases risk.

Confidentiality and documentation

HIPAA and privacy rules do not prevent required reporting through authorized channels. They do prevent gossip, social media posts, hallway discussion, and sharing details with unauthorized people. Documentation should use dates, times, direct quotes, observed facts, actions taken, and who was notified.

Final applied judgment

This final unit asks learners to apply recognition, reporting, documentation, confidentiality, and scope guardrails. The final assessment draws from all units. Certificate review remains blocked until unit activities, active participation, assessments, identity/attestation, and approval-status gates are satisfied.

Builder note: Place this lesson content in a Moodle Page or Book page with completion tracking enabled for view plus minimum expected engagement time. Keep source-module names in the builder notes and do not brand or reproduce original CCCCO/NATP source pages as learner pages.

Moodle Activity Block - H5P or Lesson Activity: Scenario/Application

Scenario: Resident L says, 'My nephew takes my bank card and yells when I ask for it back.' The resident asks you not to tell anyone and has no money for preferred hygiene items this month.

Required learner task: Final branching case: response to disclosure, reporting path, confidentiality statement, objective charting, and attestation confirmation.

Feedback design: Each incorrect path should explain the CNA scope issue, reporting delay, privacy risk, or documentation problem. Correct paths should reinforce observe, assist, report to the licensed nurse, document within scope, and protect dignity.

Moodle Activity Block - Quiz: Knowledge Check

Settings: 10 auto-graded multiple-choice items, shuffled answers, unlimited formative attempts or local policy limit, passing score 80 percent, feedback shown after attempt. Use 03_CNA_QUESTION_BANK_MOODLE_IMPORT_READY.csv for import.

Internal answer/rationale preview:

Question	Correct	Rationale
U12_Q01_source	A	U12 is mapped to Module 17 - Patient/Resident Abuse within the 12 unit x 2 hour CNA course.

Question	Correct	Rationale
U12_Q02_priority	B	The CNA observes, reports, documents, and follows policy within scope.
U12_Q03_scope	C	The listed unsafe action exceeds CNA role or violates course guardrails.
U12_Q04_charting	D	Objective documentation records facts, statements, observations, and notification.
U12_Q05_term	B	Financial abuse means misuse, theft, or coercive control of a resident's money or property.
U12_Q06_interaction	C	Active interaction requires the learner to apply judgment to the unit scenario.
U12_Q07_completion	D	The evidence model uses Moodle logs, attempts, submissions, and completion records.
U12_Q08_status	A	The course must not claim approval or issue certificates until approval status allows it.
U12_Q09_scenario	C	The first safe response is the scope-safe reporting/support action.
U12_Q10_guardrail	D	The course uses de-identified examples, online CE only, and no approval claim.

Moodle Activity Block - Assignment or Feedback: Reflection/Attestation

Prompt: Write the resident's statement as a quote, one objective observation, and the correct privacy reminder.

Attestation checkbox: I completed this unit personally, used no real resident/facility information, understand this is online CE only, and understand certificate issuance remains blocked until all course and approval gates are satisfied.

Moodle Activity Block - Completion Gate: Required Activity Completion

Completion rule: U12 is complete only when the overview page is viewed, lesson content completion is met, scenario/application activity is completed, knowledge check is passed, reflection/attestation is submitted, and any configured minimum engagement/seat-time rule is satisfied.

Evidence generated by Moodle: page views, activity completion status, time/log records, H5P or lesson attempt data, quiz attempt and score, reflection/attestation submission, and unit completion report.

Instructor/admin notes: Review local policy terms before launch. For this unit, emphasize: listen calmly, do not promise secrecy, report through required channels, and document facts.

Compliance guardrails: promise secrecy or investigate the family member independently is not allowed. Do not claim CDPH approval until issued. Do not count clinical or simulation hours. Do not copy government forms or signer binders into the course. Do not use old ContentV2 M00-M07 or an 8 x 3 structure as current.